

رئيس
المجلس الطبي المتخصص

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Name: سلوي احمد محمد حصوه National ID: 26401180102607	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 23/ 8 / 2025 G

Name	د. نوري أحمد محمد	MRN	175112025
Age	61 yrs	ID	
Menopausal status	post menopause	Phone	0102727-70
NRS SCORE		Marital status	0112498007
BMI		Family history	-ve
CO morbidity	PTN	REASON OF MDT	

Clinical Data

BX: IDC, GII

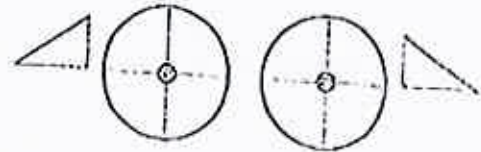
ER:	PR:	HER2:	KiTV:
7/8	8/8	(+2)	10%

Sonomammography: amplified by sish

R

MDT

LA DCE



CTNM
430

Mets Workup:

Recommendations:

- IHC Revealed that Her2 +2 (amplified by sish) [Revision]
- she Received 2 Cycles of AC.

MDT for TCHP

MDT OUTCOME & PLAN

MDT for Adjuvant Therapy
for Breast CA
TCHP X 6 → 11/24

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذ / ماري احمد محمد محمود
Patient No. : 26401180102607
Path No. : 1562/25
Sex : Female
Age : 61 Y
Referring Doc :

Request No. : 13001018
Request Date : 30/04/2025
Verification Date : 11/05/2025
Financial Category : دواءات روسي - ثالثة الدولة
Department : Out Patient
Registered by : manwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade 2.
IPT:

- E-cadherin: Positive.
- ER: Positive (score 7/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.2)).
- Ki67%: 10%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, Intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

Verified by:

DR. Menna Nabil

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المرحلة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mar El Qadima - Cairo, Egypt

Patient Name : الأستاذة / ماري أحمد محمد محمود
Patient No. : 26401180102607
Path No. : 1562/25
Sex : Female
Age : 61 Y
Referring Doc :

Request No. : 13001018
Request Date : 30/04/2025
Verification Date : 11/05/2025
Financial Category : فئة الأمانة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Amended Report

Clinical Data: Left breast locally advanced lesion.
Nature of Specimen: Tru-cut biopsy, collected on 30/4/2025 at 11.30 am.

Gross Examination:

- Multiple grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Not detected.
- Perineural invasion: Not detected.
- Angioinvasion: Not detected.
- IPT (Positive control showed appropriate reactivity):
- E-cadherin: Positive.
- ER: Moderate nuclear reaction in 70% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Faint complete membrane reaction in >10 % of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.2)).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy.

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M. F. Ghosh

ECHO REPORT

Name:

Date: 15 / 9 / 2025

EF	62%	55-75%	PWT	0.6	0.6-1.1cm
LV EDD	4.7	3.5-5.8cm	AO.R	2.5	2-3.7cm
LA ESD	2.8	2-4cm	LA	3.6	2-4cm
FS	28%	25-45%	RV		0.7-2.3cm (supine)
SWT	0.7	0.6-1.1cm	AO.CUSP		1.5-2.6cm

L.V dimensions Normal

L.V wall thickness Normal

L.V diastolic function C.I

SWM/ No ROMA

LA and AO root dimensions Normal

Mitral valve N

Aortic valve

Tricuspid valve N

Pulmonary valve

Right side and systolic function LA PSE: 2.6

Intracardiac masses and effusion

Dr. *M. F. Ghosh*

Patient Name : الأستاذ / ماري أحمد محمد محمود
Patient No. : 26401180102607
Path No. : 1562/25
Sex : Female
Age : 61 Y
Referring Doc :

Request No. : 13001018
Request Date : 30/04/2025
Verification Date : 11/05/2025
Financial Category : ثالثة الدولة - مستشفى روسي
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Prof. Dr. Maha Guimei.
Dr. Duaa Mohamed.

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DR. Menna Nabil

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حرفه رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مبنى القديسة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mir El Qadima - Cairo, Egypt

Patient ID: 81175	Patient Name: Eng. A. Silvia Ahmed Mohamed Hassan
Age: 61	2025/04/10 11:07:16 AM: Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: 61 y old lady coming with left palpable mass with skin changes.

Technique

Medio-lateral oblique and CC views. High resolution ultrasound

Detailed Findings

Left breast

- Scattered dense fibro glandular parenchyma (ACR B).
- UOQ focal asymmetry associated with distortion and overlying skin dimple seen in (CC view).
- Ultrasound demonstrated corresponding UOQ Irregular hypoechoic non parallel mass with surrounding (desmoplastic reaction), seen at 3 o'clock inner zone measuring (2.8x2cm) with anterior ductal extension reaching nipple causing nipple retraction.
- Less than 3 indeterminate axillary level 1 LNs showing eccentric hilum reaching about (0.5cm).

Right breast

- Scattered dense fibro glandular parenchyma (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast (BIRADS 4c): UOQ suspicious solid mass with nipple infiltration.

Right breast (BIRADS 1): Normal study.

Recommendations:

Biopsy was done at same sitting.

Dr. Reham Rady, FRCR, FRCR